

EMR & Digitization Advocacy Programme

Partnership Proposal — National Pilot

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01 Executive Summary

HealthStack Africa and Consult For Africa propose a joint pilot to deploy a national network of EMR and Digitization Advocates across Nigeria. These advocates will provide system-agnostic, on-the-ground support to healthcare facilities navigating the move from paper to digital.

The pilot will field **four advocates** with national remote coverage, each owning a defined geopolitical zone, supported centrally by C4A operations. The programme is structured to be commercially modest in Phase 1, prove operating mechanics over six months, then scale based on validated unit economics.

HEADLINE ASK

N13.32M for the six-month pilot covering four advocates, central coordination, and tooling. Expected output: 60 to 100 facility engagements, validated SOPs, and a scalable playbook for nationwide rollout.

02 Strategic Context

Nigeria's health digitization gap is well documented. Despite Federal mandates, donor-funded EMR deployments, and the proliferation of vendor solutions, adoption remains patchy. The reasons are not technical. They are operational, behavioural, and human.

Most facilities lack:

- A trusted advisor to guide system selection without a vendor agenda
- Hands-on support through the messy middle of implementation
- Continuity when external consultants leave or grants end
- Peer-level connection to other facilities who have made the journey

HealthStack has built one of the more credible digital health platforms in Nigeria. Consult For Africa is the leading healthcare management consulting network on the continent, with a verified pool of clinicians and healthcare operators through CadreHealth and the C4A Agent Channel.

The opportunity: combine HealthStack's product credibility with C4A's distribution and human capital infrastructure to create a category-defining advocacy programme.

03 Programme Overview

3.1 What is an EMR & Digitization Advocate?

An EMR & Digitization Advocate is a healthcare-literate, technology-fluent professional embedded within C4A's operational umbrella, deployed to engage healthcare facilities in their assigned zone. Their role is to:

1. Identify and qualify facilities ready for digitization
2. Conduct neutral, structured digital readiness assessments
3. Recommend appropriate solutions (HealthStack and others) based on facility context
4. Provide hands-on support through implementation
5. Maintain ongoing relationships post-deployment
6. Escalate to specialists when needed

Critically, advocates are positioned as **trusted advisors, not vendor representatives**. The system-agnostic stance is the differentiator and must be defended rigorously throughout the programme.

3.2 Scope of the Pilot

ELEMENT	DETAIL
Duration	6 months (May 2026 to October 2026)
Team size	4 advocates + 1 programme manager (C4A)
Geographic coverage	National, zonal allocation
Operating mode	Remote-first with regional travel
Target output	60 to 100 facility engagements
Reporting cadence	Weekly to C4A, fortnightly joint review with HealthStack

04 Operating Model

4.1 Role Split

Consult For Africa is responsible for:

- Recruitment, vetting, and onboarding of advocates
- Employment contracts, statutory compliance, and payroll
- Training curriculum design and delivery
- Day-to-day operational management
- Performance monitoring and SOP enforcement
- CRM and reporting infrastructure
- Quality assurance on facility engagements
- Monthly invoicing and financial reporting to HealthStack

HealthStack Africa is responsible for:

- Programme funding under agreed commercials
- Subject-matter input into advocate training (HealthStack product, EMR landscape)
- Lead handover of inbound HealthStack-aligned opportunities to advocates
- Strategic direction and quarterly programme review
- Co-branding approval and marketing alignment

4.2 Why C4A Manages the Operating Layer

Healthcare workforce deployment is operationally heavy. Recruiting, contracting, training, performance-managing, and paying field personnel across Nigeria is a discipline in its own right. C4A's CadreHealth platform already handles this for clinical placements. Layering advocacy on top is a natural extension.

It also preserves HealthStack's positioning. The advocacy programme is not a HealthStack sales channel. It is a neutral healthcare advisory function that happens to surface HealthStack as one option among many. That neutrality is strongest when the operating entity is not HealthStack itself.

05 Advocate Role Definition

5.1 Position Profile

FIELD	DETAIL
Job Title	EMR & Digitization Advocate
Reports To	Programme Manager (C4A)
Engagement Type	12-month consulting contract via Consult For Africa
Location	Remote, with assigned zone of responsibility

5.2 Core Responsibilities

- Discovery and outreach.** Identify priority facilities in assigned zone using CadreHealth data, public records, and partner referrals. Conduct first-contact engagements via phone, video, or in-person where warranted.
- Digital readiness assessment.** Run a standardized 60 to 90 minute structured assessment covering: current workflows, infrastructure, staff readiness, leadership commitment, budget posture, and existing systems.
- Recommendation and roadmapping.** Produce a written report for the facility outlining strengths, gaps, and a phased pathway. Include a shortlist of two to three system options matched to facility profile. HealthStack appears where genuinely fit-for-purpose.
- Implementation support.** Where the facility selects a path forward, provide structured hand-holding through vendor engagement, configuration choices, training, and go-live. The advocate does not implement systems. They orchestrate.
- Post-deployment continuity.** Maintain quarterly check-ins with engaged facilities for at least 12 months. Surface adoption issues, escalate to vendors, and track outcomes.
- Knowledge contribution.** Submit weekly field intelligence to C4A. Participate in monthly cohort calls. Contribute to evolving SOPs.

5.3 Required Profile

- Healthcare background (clinical or hospital management) with 3+ years experience
- Demonstrated comfort with digital tools and systems thinking
- Strong written and verbal communication in English
- Local language fluency relevant to assigned zone (Hausa, Yoruba, Igbo, or Pidgin)

- Self-directed, mobile, and willing to travel within zone
- Reference-checkable and license-verified through CadreHealth

5.4 Disqualifying Affiliations

To preserve neutrality, advocates may not:

- Hold equity, stipends, or referral arrangements with any EMR or HIS vendor
- Concurrently work as a sales agent for any health technology product
- Receive personal compensation from vendors during their tenure

This will be enforced through contract clauses, periodic attestations, and random audit.

06 Coverage Strategy

6.1 Zonal Allocation (4 Advocates)

ADVOCATE	ZONE	STATES COVERED	EST. FACILITIES
1	South West + part South South	Lagos, Ogun, Oyo, Ondo, Osun, Ekiti, Edo, Delta	280+
2	South East + rest South South	Imo, Anambra, Enugu, Abia, Ebonyi, Cross River, Akwa Ibom, Bayelsa, Rivers	220+
3	North Central including FCT	FCT, Plateau, Benue, Kogi, Kwara, Niger, Nasarawa	180+
4	North West + North East	Kano, Kaduna, Katsina, Sokoto, Kebbi, Jigawa, Zamfara, Bauchi, Borno, Adamawa, Gombe, Taraba, Yobe	240+

6.2 Travel Posture

Default operating mode is remote (phone, video, structured digital assessments). Physical site visits are reserved for:

- Facilities that have signalled high readiness and require trust-building
- Implementation kickoff for facilities adopting any platform
- Quarterly tier-1 facility reviews

Each advocate will have a quarterly travel budget to support up to four in-zone trips per month, scaling based on engagement volume.

07 Standard Operating Procedures

7.1 SOP 1: Facility Identification and Qualification

Trigger: Weekly territory planning session.

Steps:

1. Pull list of priority facilities in zone from CadreHealth facility database (filtered by bed count, ownership type, prior engagement status).
2. Cross-reference with publicly available digitization signals (state grant lists, JCI/COHSASA pipeline, NHIA partnership status).
3. Score and rank using the C4A Digital Readiness Lead Score (proprietary).
4. Submit weekly engagement plan to Programme Manager every Monday by 12:00 WAT.

Output: Ranked list of 8 to 12 facilities for the week's outreach.

7.2 SOP 2: First Contact and Engagement Booking

Trigger: Facility appears on weekly engagement plan.

1. Identify decision-maker (Medical Director, Hospital Administrator, IT lead, or owner).
2. Reach out via the C4A-approved channel (warm referral, LinkedIn, association introduction, or cold WhatsApp).
3. Use the standardized first-contact script (see Appendix A).
4. Book a 60-minute discovery call within 14 days.
5. Log every contact attempt in CadreHealth CRM.

Service standards: No more than three follow-up attempts per facility within a four-week window. After that, mark as cold and move on.

7.3 SOP 3: Digital Readiness Assessment

Trigger: Discovery call confirmed.

1. Send pre-call briefing pack 48 hours in advance.
2. Conduct structured 60 to 90 minute assessment using the C4A Digital Readiness Framework.
3. Score across six dimensions: workflows, infrastructure, staff, leadership, budget, current systems.
4. Within 5 business days, deliver a written assessment report (10 to 15 pages) including findings, score, and prioritized recommendations.
5. Schedule follow-up to walk through the report.

Quality gate: Programme Manager reviews and approves every assessment report before delivery to facility.

7.4 SOP 4: Solution Recommendation

Trigger: Assessment complete, facility ready to consider options.

1. Match facility profile to the C4A Solution Library (maintained by Programme Manager, includes HealthStack and competing systems).
2. Identify two to three viable solutions across price points.
3. Present options in a single comparison document. Never recommend a single system. Always present alternatives.
4. Disclose any C4A affiliation with vendors in the recommendation footer.
5. Hand off to facility for vendor engagement.

Neutrality safeguard: Quarterly external audit of recommendation patterns. If any single vendor exceeds 50% of recommendations, trigger root-cause review.

7.5 SOP 5: Implementation Support

Trigger: Facility selects a vendor.

1. Co-develop a phased implementation plan with vendor and facility (12 to 16 weeks typical).
2. Attend vendor kickoff meeting.
3. Conduct fortnightly check-ins with facility through go-live.
4. Surface and escalate blockers to Programme Manager and vendor.
5. Conduct 30, 60, and 90-day post-go-live adoption reviews.

7.6 SOP 6: Reporting and Cadence

CADENCE	ACTIVITY	OWNER
Daily	Activity log in CadreHealth CRM	Advocate
Weekly	Pipeline update to Programme Manager	Advocate
Weekly	Cohort sync, 45 min, all advocates	Programme Manager
Fortnightly	Joint review with HealthStack	PM + HealthStack
Monthly	Performance dashboard to all stakeholders	Programme Manager
Quarterly	Strategic review and SOP iteration	Joint leadership

7.7 SOP 7: Escalation Matrix

ISSUE TYPE	FIRST RESPONDER	ESCALATION PATH
Facility complaint	Advocate within 24h	PM within 48h
Vendor non-performance	Advocate logs in CRM	PM engages vendor
Conflict of interest	Programme Manager	Joint partnership leadership
Compliance breach	Programme Manager	C4A Partner + HealthStack Founder
Safety or clinical risk	Advocate within 4h	C4A Partner + facility leadership

08 Recruitment & Selection

8.1 Pipeline

C4A will source advocates from three pools:

1. **CadreHealth verified pool.** 4,000+ professionals across cadres, license-verified.
2. **Doctors Foundation for Care alumni.** 500+ diaspora-connected clinicians and admins.
3. **Open application.** Public posting through CadreHealth jobs board and the C4A Agent Channel.

8.2 Selection Process

1. Application screen
2. Structured competency interview (C4A)
3. Joint final-round panel (C4A + HealthStack)
4. Reference check and license verification
5. Offer and contract

Target time-to-hire: 4 weeks from programme greenlight.

09 Training & Certification

A 10-day induction programme delivered jointly by C4A and HealthStack:

MODULE	DURATION	DELIVERY
Healthcare digital landscape Nigeria	1 day	C4A
EMR/HIS technology fundamentals	2 days	HealthStack
C4A Digital Readiness Framework	2 days	C4A
Vendor landscape and product comparison	1 day	HealthStack + C4A
Field SOPs and CRM workflows	1 day	C4A
Engagement skills and consultative selling	1 day	C4A
Compliance, ethics, and neutrality	1 day	C4A
Field shadowing and case clinics	1 day	Joint

Advocates must pass a certification assessment before being deployed. Recertification every six months.

10 Performance Management & KPIs

10.1 Per-Advocate KPIs

METRIC	MONTHLY TARGET	NOTES
Facilities contacted	20 to 30	Quality over quantity
Discovery calls completed	8 to 12	At least 60-min structured
Assessments delivered	4 to 6	Quality-gated
Implementation engagements active	2 to 4	Cumulative
90-day adoption review rate	80%+	Of facilities that go live
CRM hygiene score	95%+	Audited monthly

10.2 Programme-Level KPIs (6 months)

- 60 to 100 facility engagements
- 30 to 50 assessments delivered
- 10 to 20 implementations supported (any vendor)
- Net Promoter Score from facilities: 40+
- Programme cost per facility engaged: under N50k

10.3 Performance Reviews

- 30-day post-induction probation review
- Quarterly performance reviews
- Underperformance triggers a 30-day improvement plan
- Repeated failure to meet KPIs results in offboarding

11 Commercial Structure

11.1 Pilot Commercials (6 months)

Per advocate, monthly:

ITEM	AMOUNT (NGN)
Base retainer	250,000
Travel and field allowance	60,000
Communications and tooling	20,000
Performance bonus pool (capped)	up to 70,000
Total per advocate / month	up to 400,000

Programme-level monthly:

ITEM	AMOUNT (NGN)
4 advocates fully loaded	1,600,000
Programme Manager (C4A, 50% allocation)	250,000
CRM, training platform, analytics	80,000
C4A management fee (15%)	290,000
Total monthly run rate	2,220,000

PILOT TOTAL

N13.32M for 6 months, paid in three tranches: 40% upfront, 30% at month 3, 30% at month 6.

11.2 Performance Bonus Mechanics

The performance bonus pool of up to N70k per advocate per month is unlocked based on:

- Facility engagement volume (40%)
- Assessment quality score from PM audit (30%)
- 90-day adoption review completion (20%)
- Cohort contribution (10%)

This creates upside without distorting neutrality.

11.3 Optional Variable Layer (Phase 2)

If the pilot succeeds, Phase 2 can layer in a per-facility-onboarded bonus paid by HealthStack only when a facility chooses HealthStack. To preserve neutrality, this would be paid into a programme bonus pool (split equally among all four advocates regardless of who closed), not directly to the closing advocate.

12 Governance & Reporting

12.1 Joint Steering Committee

- Quarterly meetings
- Members: HealthStack Founder, C4A Partner, Programme Manager, one advocate (rotating)
- Agenda: KPI review, budget tracking, strategic direction, escalations

12.2 Monthly Performance Pack

C4A delivers to HealthStack on the 5th of each month:

- Activity dashboard (per advocate, per zone)
- Pipeline analysis
- Win/loss review
- Field intelligence summary
- Financial reconciliation
- Risk and issue log

12.3 Data and IP

- Anonymized facility intelligence is shared in aggregate
- Identified facility data requires facility consent before sharing
- SOPs, training material, and CRM templates are jointly owned
- The C4A Digital Readiness Framework remains C4A intellectual property, licensed to the programme

13 Timeline

MONTH	MILESTONES
Month 0 (April)	Sign partnership agreement, kick off recruitment
Month 1 (May)	Hire 4 advocates, complete induction, deploy to zones
Months 2 to 3	Pilot phase 1: outreach and assessments
Month 3 review	First quarterly review, SOP refinement
Months 4 to 5	Pilot phase 2: implementations and continuity
Month 6 (October)	Pilot close-out review, commercial decision on Phase 2

14 Risks and Mitigations

RISK	LIKELIHOOD	IMPACT	MITIGATION
Advocates drift toward HealthStack as default recommendation	Medium	High	Quarterly recommendation audit, contract clauses, neutrality KPI
Slow facility engagement in target months	Medium	Medium	Conservative KPIs, pivot to government channel if private market resistant
Advocate attrition	Medium	Medium	Strong vetting, cohort structure, performance bonus, 12-month commitment
Funding interruption	Low	High	Tranched payments, monthly invoicing, 30-day notice clauses
Vendor relations strained by neutrality	Low	Medium	Transparent positioning, public framework, vendor briefings
Reputational risk to HealthStack from advocate behaviour	Low	High	C4A QA, escalation matrix, contractual indemnities

15 Why This Will Work

This is not a sales programme dressed up as advisory. It is genuinely structured to be neutral, which is precisely what creates the strategic value for HealthStack. Facilities are tired of being sold to. They are starved for trustworthy guidance.

When advocates earn their trust, three things follow:

1. Facilities self-select toward the systems best fit for them (HealthStack often wins on merit)
2. Adoption rates rise because the implementation discipline is enforced by a neutral party
3. The programme generates field intelligence that informs HealthStack product roadmap

Six months is enough time to prove this. Four advocates is the minimum viable footprint for national signal. C4A has the people, processes, and platform to make it happen.

16 Next Steps

If aligned in principle:

1. Two-hour working session to refine commercials and finalise zonal allocation (within 7 days)
2. Sign Memorandum of Understanding (within 14 days)
3. Begin recruitment immediately, target deployment by 1 May 2026
4. Co-announce the programme externally at month 2

We are ready to move. Look forward to your response.

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A Appendix A: First Contact Script

To be developed jointly during induction. Below is a draft of the script and key talking points.

"Good afternoon, this is [Name] calling from Consult For Africa. We are running a national programme to support hospitals through the digitization journey, regardless of which system you eventually choose. Most facilities we speak with are weighing options, dealing with vendor pressure, or trying to make sense of where to start. We provide free initial consultations and work as your independent advisor through the process. Would you have 30 minutes next week for a structured conversation about where your facility currently sits on this journey?"

Key principles:

- Lead with neutrality
- Offer the consultation as free in pilot phase
- Position as advisor, never as salesperson
- Defer questions about specific systems to the assessment

B Appendix B: Digital Readiness Framework

The six assessment dimensions:

1. **Workflows.** Patient registration, clinical documentation, billing, pharmacy, lab, imaging, discharge. Where is paper, where is digital, where is hybrid.
2. **Infrastructure.** Internet reliability, power, hardware fleet, network, security baseline.
3. **Staff readiness.** Digital literacy distribution, change appetite, prior system experience, training infrastructure.
4. **Leadership commitment.** Executive sponsorship, board awareness, budget allocation, change management capacity.
5. **Budget posture.** Capital available, operating expense tolerance, willingness to consider subscription models, donor or grant context.
6. **Current systems.** What exists, what is used, what was abandoned, vendor relationships, data exportability.

Each dimension scores 0 to 10. Composite score determines readiness band: Not Ready, Foundational, Implementation Ready, Optimization, Advanced.

End of Proposal

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